

ADVANCED LEVEL

MULTI-COMPOUND

16-24 WEEKS

GH + PEPTIDES

ADVANCED ANABOLIC CYCLE MASTERY

Elite Multi-Compound Strategies, Competition Prep & GH Integration

Advanced Stacks • Competition Protocol • Peptide Integration • Health Monitoring

ROYAL FITNESS CLUB — ANABOLIC SERIES

Anabolic Series Vol. 3 • Advanced Edition • 2024

DISCLAIMER: Educational purposes only. This guide discusses advanced pharmacological protocols for academic and harm-reduction purposes only. Consult a licensed physician before any hormonal intervention.

■ Advanced Anabolic Concepts

Concept	Definition	Practical Application
Androgen Receptor Upregulation	Increase in AR density with sustained exposure	Blast/cruise allows continuous receptor sensitivity
Androgen Receptor Downregulation	Decreased sensitivity with continuous high-dose use	Why 16-week cycles outperform 24-week cycles per mg
Synergistic Stacking	Compounds with complementary mechanisms amplify each other	Test + Tren + Mast = DHT-style hardening synergy
Progestin Activity	Nandrolone/Tren raise progesterone — interacts with prolactin	Cabergoline management critical — can cause gyno without E2 elevation
5 α -Reduction	DHT conversion in skin/scalp/prostate	Finasteride blocks 5 α -reductase — reduces hair/prostate risk (not with Tren)
HPTA Suppression Depth	Higher doses + longer cycles = deeper suppression	Recovery harder after long blasts — TRT bridge sometimes necessary
Blast & Cruise	Alternating high-dose cycles with TRT-level maintenance	Avoids repeated PCT — common in competitive bodybuilders

■ Competition Prep Stack — 20-Week Protocol

This protocol mirrors a competitive bodybuilding prep cycle. It is high-risk, high-reward and requires extensive experience, monitoring, and medical supervision.

Phase	Duration	Compounds	Primary Goal
Off-Season Base	Year-round (blast)	Test-E 600mg + Deca 400mg + HGH 2–4 IU	Maximise muscle mass accumulation
Early Prep (Wks 1–8)	8 weeks	Test-E 400mg + Tren-E 300mg + Anavar 50mg + HGH 4IU	Begin fat loss while preserving mass
Peak Prep (Wks 9–16)	8 weeks	Test-P 200mg + Tren-A 400mg + Mast-P 400mg + Anavar 80mg	Maximum hardness, vascular, fat mobilisation
Peak Week (Wks 17–20)	4 weeks	Test-P 100mg + Tren-A 300mg + Mast-P 400mg + Winstrol 50mg	Final conditioning — water manipulation
Post Show PCT	6–8 weeks	Nolvadex 40/40/20/20mg + Clomid 50/50/25/25mg	Full HPTA recovery

Compound	Dose	Weeks	Mechanism	Side Effect Focus
Testosterone Propionate	100–200 mg	1–20	Anabolic base — short ester for quick removal	Low — base hormone
Trenbolone Enanthate	300 mg	1–8	Strong anabolic/androgenic, fat loss	Night sweats, prolactin, CV strain
Trenbolone Acetate	300–400 mg	9–20	Switch to short ester for show-week control	Same as above + tren cough
Masteron Propionate	300–400 mg	9–20	Anti-oestrogenic, hardening, DHT derivative	Hair loss if genetically prone
Anavar	50–80 mg	1–16	Lean mass, strength, minimal water retention	Hepatotoxic — use TUDCA
Winstrol	50 mg	17–20	Extreme hardening, SHBG reduction	Joint pain, hepatotoxic, HDL crash
HGH (somatropin)	2–4 IU	Year-round	IGF-1 elevation, fat oxidation, recovery	Carpal tunnel, water retention, cost
Insulin (Humalog)	4–8 IU post-training	Advanced only	Nutrient partitioning, glycogen	Hypoglycaemia risk — EXTREME caution

■■ **IMPORTANT:** Insulin use is extremely dangerous. A miscalculation can cause fatal hypoglycaemia within minutes. This is included for educational awareness ONLY. Never use insulin without direct medical supervision.

■ Human Growth Hormone (HGH) — Complete Protocol

HGH Protocol	Dose	Timing	Goal	Notes
Anti-ageing / Wellness	1–2 IU	AM fasted	Health optimisation, fat loss	Low dose — minimal side effects
Bodybuilding (offseason)	2–4 IU	Pre-workout or post-training	Muscle gain + fat loss balance	Split into 2 doses if using 4 IU
Aggressive bodybuilding	4–6 IU	2 IU AM + 2–4 IU pre-training	Maximum anabolic effect	High cost, side effect risk rises
Competition prep	4–8 IU	AM + post-training	Fat loss + conditioning	Often paired with insulin — extreme risk
Peptide alternative (GHRP-2)	100–200 mcg	3x daily, fasted	GH release stimulation	Legal, cheaper, less effective
Peptide alternative (CJC-1295)	100 mcg	2x daily	Sustained GH pulse elevation	Stack with GHRP for synergy

HGH Side Effect	Threshold	Management
Water retention	All doses	Reduces after 4–6 weeks of use
Carpal tunnel syndrome	2+ IU/day	Reduce dose, wrist splints, usually reverses on cessation
Acromegaly (long-term)	Supraphysiological long-term use	Avoid excessive dose for extended periods
Insulin resistance	4+ IU/day	Monitor glucose, use metformin if needed
Hypothyroidism	Long-term use	T3/T4 monitoring — may need T3 supplementation
Increased IGF-1	Dose-dependent	Acceptable elevation — excessively high IGF-1 has cancer associations

■ Blast & Cruise Protocol (B&C;) — Advanced Continuous Approach

Blast & Cruise eliminates PCT between cycles. During the 'cruise' phase, the user drops to TRT-level testosterone (100–150mg/week), maintaining hormonal function while allowing partial recovery before the next blast.

Phase	Duration	Compound	Dose	Goal	Health Monitoring
BLAST 1	16 weeks	Test-E + Deca + Anavar	500+400+50mg	Mass gain	Bloods at wk 6 + wk 16
CRUISE 1	8–12 weeks	Test-E only	100–150 mg/week	Recovery + maintenance	Bloods at wk 4 into cruise
BLAST 2	14 weeks	Test-E + Tren-A + Mast-P	400+300+300mg	Recomp / lean mass	Bloods at wk 6
CRUISE 2	8 weeks	Test-E only	100–150 mg/week	Recovery	Full hormone panel
BLAST 3	12 weeks	Test-P + Tren-A + Mast + Var	200+400+400+60mg	Competition prep	Weekly BP, bloods at wk 6

■ *Blast & Cruise is a permanent commitment to exogenous testosterone. Fertility is severely compromised. If you wish to father children, HCG (500 IU 2x/week) throughout all phases is essential to maintain testicular function.*

■ Advanced Health Monitoring Schedule

Test	Frequency	Critical Thresholds	Action if Exceeded
Blood Pressure	Weekly	Systolic >140 / Diastolic >90	Reduce dose, add telmisartan, increase cardio
Haematocrit	Every 6 weeks	Above 54%	Blood donation, increase hydration, reduce AAS dose
HDL Cholesterol	Every 6 weeks	Below 35 mg/dL	Reduce orals, increase omega-3, add cardio, check statin
LDL Cholesterol	Every 6 weeks	Above 160 mg/dL	Diet intervention, reduce orals, consider rosuvastatin
Liver Enzymes (AST/ALT)	Every 6 weeks	3× upper normal limit	Stop all orals immediately, use TUDCA, retest in 4 weeks
Prolactin	Every 8 weeks on Tren/Deca	Above 30 ng/mL	Cabergoline 0.5mg 2x/week
PSA	Every 3–6 months (35+)	Rapidly rising or >4.0 ng/mL	Urologist referral immediately
Full Cardiac Echo	Annually	LV wall thickness >13mm	Cardiology referral — LVH risk with long-term AAS

■ Advanced PCT — Recovery After Multi-Compound Cycles

Cycle Type	Clearance Wait	PCT Protocol	Duration	Recovery Expectation
Test-E only	2 weeks	Nolvadex 40/40/20/20mg	4 weeks	Full in 4–8 weeks
Test + Deca	3 weeks (Deca long ester)	Nolva 40/40/20/20 + Clomid 50/50/25/25	6 weeks	8–12 weeks — Deca deeply suppressive
Test + Tren-A	3 days (Tren Ace short ester)	Nolva 40/40/20/20 + Cabergoline during PCT	4–6 weeks	6–10 weeks — Tren very suppressive
Test + Tren-E	3 weeks	Nolva 40/40/20/20 + Clomid + Caber	6 weeks	10–16 weeks
B&C; (returning to natural)	3+ weeks from last blast	Nolva 40/40/20/20 + Clomid 50/50/25/25 + HCG 1000 IU EOD × 3 weeks	8–10 weeks	12–24 weeks — may not fully recover
Post-competition (full prep)	2–3 weeks	Nolva 40/40/20/20 + Clomid + HCG + DHEA	8+ weeks	16–24 weeks — expect low mood, low T initially

PCT Support Supplement	Dose	Role	Duration
HCG (pre-PCT only)	1000 IU EOD × 10 days	Directly stimulates Leydig cells — jumpstarts testosterone production	Use BEFORE starting SERM
Nolvadex (Tamoxifen)	40→20 mg/day tapering	Blocks oestrogen at pituitary — restores LH/FSH release	4–6 weeks
Clomid (Clomiphene)	50→25 mg/day	Competitive E2 blocker — amplifies LH pulse	4–6 weeks
Cabergoline	0.25 mg 2x/week	Lowers prolactin post-Deca/Tren — facilitates recovery	During PCT if levels elevated
Enclomiphene (modern alternative)	12.5–25 mg/day	Selective isomer of Clomid — fewer side effects, strong LH stimulus	4 weeks
Ashwagandha (KSM-66)	600 mg/day	Reduces cortisol, supports natural T recovery	Throughout PCT
D-Aspartic Acid	3g/day	Short-term natural LH stimulation	4–8 weeks
Zinc + Magnesium (ZMA)	Zinc 30mg + Mag 450mg	Testosterone cofactors — support enzymatic T synthesis	Indefinitely